

Table 1: Two-Way Fixed Effects Estimates of *post* on Selected Quality Metrics

Metric	w/o staffing as a control	w/ staffing as a control
Labor Saving Mechanisms		
Catheter	−0.173*** (0.050)	−0.175*** (0.051)
Antipsychotic	−0.116 (0.218)	−0.134 (0.218)
Hypnotics	−0.439** (0.211)	−0.421* (0.212)
Outcomes		
Pressure injuries	0.148 (0.179)	0.159 (0.181)
Falls with major injury	−0.009 (0.067)	−0.007 (0.067)
Weight Loss	0.102 (0.114)	0.095 (0.113)
ADL Increase	0.228 (0.257)	0.254 (0.262)
Urinary Tract Infections	−0.310*** (0.076)	−0.302*** (0.076)

Notes: Each cell reports the coefficient on *post* with two-way clustered standard errors (by facility and quarter) in parentheses.

All specifications exclude observations with $\tau = 0$.

The first column of estimates includes standard controls only. The second column additionally includes RN, LPN, and CNA HPRD as staffing controls.

Standard controls: government + non_profit + chain + beds + occupancy_rate + pct_medicare + pct_medicaid + cm_q_state_2 + cm_q_state_3 + cm_q_state_4.

Staffing-controlled specification: government + non_profit + chain + beds + occupancy_rate + pct_medicare + pct_medicaid + cm_q_state_2 + cm_q_state_3 + cm_q_state_4 + rn_hprd + lpn_hprd + cna_hprd.

Outcome-specific windows and estimation sample sizes: Catheter [2017Q1–2024Q2; N=197,438]; Antipsychotic [2017Q1–2024Q2; N=195,998]; Hypnotics [2017Q1–2024Q2; N=196,555]; Pressure injuries [2018Q1–2023Q3; N=139,764]; Falls with major injury [2017Q1–2024Q2; N=200,132]; Weight Loss [2017Q1–2024Q2; N=195,249]; ADL Increase [2017Q1–2024Q2; N=187,542]; Urinary Tract Infections [2017Q1–2024Q2; N=199,311].

Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.